

Escitalopram Taper & MDMA-AT Preparation Protocol

Starting point: 20 mg escitalopram, ~7-8 years of continuous use **End point:** Off escitalopram with adequate washout for MDMA-assisted therapy **Default duration:** ~5-6 months taper + 2-3 months washout = ~8-9 months total **Backup duration if she runs into trouble:** ~12-18 months taper (see "If symptoms get bad" sections)

How this protocol works

This is a **standard-pace stepwise taper** with explicit decision points. After each reduction, she and her prescriber check how she's doing. If she's stable, she continues at this pace. If she's struggling, she slows down — sometimes a little, sometimes a lot.

Most long-term escitalopram users get through a taper at this pace without major issues. Some don't, and the structure below is designed to detect that early and adapt rather than push through.

This is not self-administered. It's a discussion document for her prescriber.

Phase 0 — Setup (Weeks -2 to 0)

Two weeks of preparation before any dose change.

Prescriber visit. Confirm willingness to support a taper over ~5-6 months with the option to slow down. Get a script for **escitalopram oral solution (1 mg/mL)** to have on hand for the lower doses (you don't need it yet, but it's annoying to need it on a Saturday and not have it). Discuss the MDMA-AT goal so the timeline makes sense to them.

Start a simple daily log. Sleep (1-10), mood (1-10), anxiety (1-10), any physical symptoms. Two weeks of "what does normal feel like" data is invaluable for telling withdrawal from relapse later. A notes app works fine — doesn't need to be elaborate.

Stabilize the basics. Regular sleep, moderate exercise, minimal alcohol. Identify a therapist if she doesn't have one — ideally someone she'd want to do MDMA integration work with eventually. Don't start a taper the week before a major life event.

Tell 1-2 trusted people so they can flag if she seems off. Withdrawal is sometimes more visible to others than to oneself.

Phase 1 — Standard reductions (Weeks 1-20)

Four steps, four weeks each, ~25-30% reductions of the previous dose. This is roughly what a thoughtful prescriber would propose as the default plan.

Weeks 1-4: 20 mg → 15 mg

How: 1.5×10 mg tablets, or 1×10 mg + 1×5 mg tablet.

What's normal: Mild dizziness, slightly weird dreams, brief irritability for 5-10 days after the change. Some people have nothing.

What's a yellow flag: Brain zaps that don't ease by day 14. Sleep disrupted more than 2 nights/week. Anxiety up enough to interfere with work.

What's a red flag: Severe symptoms (akathisia, panic attacks lasting hours, dark intrusive thoughts, can't function). → Back up to 20 mg, stabilize for 4 weeks, then try $20 \rightarrow 17.5 \rightarrow 15$ over 8 weeks instead.

Decision point at week 4: If she's mostly fine, continue. If yellow flags, hold at 15 mg for an extra 2-4 weeks before next step. If red flags, see above.

Weeks 5-8: 15 mg → 10 mg

How: 1×10 mg tablet.

What's normal: Similar profile to the first step. Brain zaps may show up if they didn't before.

Decision point at week 8: Same logic — stable, continue; rough, hold; severe, back up.

Weeks 9-14: 10 mg → 5 mg (extra 2 weeks built in)

How: 1×5 mg tablet.

Why the extra time: This is a bigger proportional jump than it looks. Going from 10 to 5 mg is roughly a 50% reduction in serotonin transporter occupancy because of the curve. This is the step where standard tapers most often start producing trouble.

What's normal: Brain zaps, sleep disruption, transient mood dips, some emotional volatility for 1-2 weeks.

What's a yellow flag: Symptoms still daily at week 12. Hold at 5 mg for an extra 2-4 weeks.

What's a red flag: Same as before. → Back up to 7.5 mg (use liquid: 7.5 mL), stabilize for 4 weeks, then $7.5 \rightarrow 6 \rightarrow 5$ in two slower steps.

Weeks 15-20: 5 mg → 2.5 mg (extra 2 weeks built in)

How: Half a 5 mg tablet (use a blade cutter), or 2.5 mL of liquid.

Why the extra time: Same reason — the curve is getting steep.

Decision point at week 20: This is a major checkpoint. How is she doing overall? If smoothly, the standard final step is reasonable. If she's been holding extra weeks at every step or had to back up, switch to the slower finale below.

Phase 2 — The finale (Weeks 21–24, or longer)

This is where the standard pace and the slow pace diverge.

Standard finale (if Phase 1 went smoothly)

Weeks 21–24: 2.5 mg → 0 mg

Two options, equally reasonable:

- **Direct stop after 4 weeks at 2.5 mg.** Some people do this without issue.
- **Brief intermediate step:** 2.5 mg → 1 mg (1 mL liquid) for 2 weeks → 0 mg. Slightly more cautious.

What's normal: A wave of withdrawal symptoms 5–14 days after the last dose, easing over 2–3 weeks. Brain zaps, mood lability, sleep disruption.

Slow finale (if Phase 1 was bumpy)

If she had to hold extra weeks repeatedly, back up at any point, or if she generally feels fragile heading into this phase, do not try to finish in 4 weeks. Instead:

Weeks 21–24: 2.5 → 1.5 mg (1.5 mL liquid) **Weeks 25–28:** 1.5 → 1 mg (1 mL liquid) **Weeks 29–32:** 1 → 0.5 mg (0.5 mL liquid) **Weeks 33–36:** 0.5 → 0.25 mg (0.25 mL liquid) **Weeks 37–40:** 0.25 → 0 mg

This adds about 4 months. It's the right call if her body has been telling her the curve is steep for her — and it's still much shorter than the maximally cautious 17-month version.

Phase 3 — Off escitalopram, stabilizing (Months 6–8 standard, 10–12 if slow finale)

The first 2–3 months off are when most acute withdrawal resolves. **This is not the time to start MDMA**, even if she feels great.

What to expect:

- "Windows and waves" — periods of feeling good alternating with rough patches. Waves typically shorten over weeks.

- Emotional reactivity returning. Things feel sharper — both pleasant and unpleasant. Many people find this welcome after years of muted feeling.
- A baseline emerging that may look different from her pre-medication baseline (it's been 7-8 years).

Red flag for relapse vs. withdrawal: If by month 2-3 off she has persistent, classical depression — flat affect, anhedonia, hopelessness, sleep/appetite changes — sustained for 3+ weeks without improvement, that's the conversation to have with her prescriber. Withdrawal tends to be lumpy and improving over time; relapse tends to be steady and worsening.

What helps during this phase:

- Continued daily logging
 - Regular therapy
 - Sleep, exercise, social contact, reduced alcohol
 - Patience — people often feel notably better at month 6 than month 2
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Phase 4 — MDMA preparation (Months 7-9 standard timeline)

The serotonin system upregulates over time after long-term SSRI exposure. **For MDMA to work therapeutically, not just be safe, this recovery time matters.**

The minimum safe washout is ~2 weeks (5 half-lives). The minimum *effective* washout is longer — most experienced clinicians want **2-3 months minimum** off, and after 7-8 years of use, leaning toward 3 months is the conservative call.

During this phase:

- Engage actively with the MDMA-AT pathway: clinical trial, Australian program, etc.
- Most legitimate programs require confirmation she's been off serotonergic medications for their specified window
- Begin or deepen preparation work with her therapist:
 - Intentions for the MDMA work
 - Trauma history review if relevant
 - Somatic awareness practices
- Confirm integration support is lined up for after sessions

48-72 hours before any MDMA session: Review all supplements. 5-HTP, St. John's Wort, high-dose tryptophan, certain mushroom supplements are contraindicated. So is tramadol if she's prescribed anything for pain.

Universal hold/back-up rules

Hold (don't take next step) if at end of a 4-week step:

- Withdrawal symptoms still daily and noticeable
- Sleep disrupted >2 nights/week
- Anxiety/mood interfering with daily function
- Major life stressor has emerged

Back up to previous dose if:

- Symptoms severe (7+/10 on her tracking)
- Akathisia (driven inner restlessness, can't sit still) — this one warrants same-day prescriber contact
- Suicidal thoughts emerge or intensify
- Can't function at work/relationships

Backing up is a normal part of the process, not failure. After 4 weeks stabilizing at the previous dose, retry the same step more slowly (split it in two).

Same-day prescriber contact for:

- Suicidal thoughts with intent or planning
 - Akathisia
 - Severe panic attacks lasting hours
 - Symptoms preventing eating, sleeping, or working >3 days
 - Anything that frightens her
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Tracking template

Date: _____
Current dose: ____ mg
Days since last reduction: ____

Sleep last night: ____/10 | Hours: ____

Mood today: ___/10

Anxiety today: ___/10

Energy: ___/10

Physical symptoms (circle): brain zaps / dizziness / nausea /
headache / GI / vivid dreams / other: _____

Notes: _____

The pattern in the days right after a dose change is what matters most. Weekly review is enough.

At-a-glance schedule

Standard pace (default)

Phase	Weeks	Dose path	Months
Setup	−2 to 0	20 mg	—
1	1-20	20 → 15 → 10 → 5 → 2.5	M1-M5
2	21-24	2.5 → 0	M6
3	M6-M8	Off, stabilizing	—
4	M7-M9	MDMA prep & sessions	—

Slow pace (if needed)

Phase	Weeks	Dose path	Months
Setup	−2 to 0	20 mg	—
1	1-20	20 → 15 → 10 → 5 → 2.5 (with holds)	M1-M5
2	21-40	2.5 → 1.5 → 1 → 0.5 → 0.25 → 0	M6-M10
3	M10-M13	Off, stabilizing	—
4	M12-M14	MDMA prep & sessions	—

Resources

- **Maudsley Deprescribing Guidelines** (Horowitz & Taylor, 2024) — the reference text on hyperbolic tapering
- **Outro Health** (outro.com) — clinician-supported deprescribing in select US states
- **Surviving Antidepressants** (survivingantidepressants.org) — peer support; useful but skews toward severe withdrawal experiences, read with discernment
- **MAPS / Lykos** — current MDMA-AT trial info
- **ClinicalTrials.gov** — search "MDMA" + her diagnosis

Discussion starting point with her prescriber. Modify based on her actual response.